

Ofcin F. C. Tablet

Ingredient(s):

Ofcin F. C. Tablet 200mg:

Each tablet contains:

Ofloxacin 200mg

Ofcin F. C. Tablet 400mg:

Each tablet contains:

Ofloxacin 400mg

Pharmacology (Summary of Pharmacodynamics and Pharmacokinetics):

Pharmacodynamics

1. Ofloxacin is bactericidal in action. Like other quinolones, it inhibits DNA gyrase, an enzyme necessary for bacterial DNA replication, in susceptible organisms. Inhibition of DNA gyrase results in inhibition of ATP-dependent negative supercoiling of DNA, inhibition of ATP-independent relaxation of supercoiled DNA, and promotion of double-stranded DNA breakage.
2. It is active in vitro against many Gram positive aerobic bacteria, including penicillinase producing, non-penicillinase producing, and some methicillin resistant staphylococci. It is also active in vitro against most Gram negative aerobic bacteria, including Enterobacteriaceae and Pseudomonas aeruginosa. Like other fluoroquinolones, it is generally less active against gram positive than Gram negative bacteria. It also has some activity in vitro against Chlamydia, Mycoplasma, Mycobacterium, Plasmodium and Rickettsia.
3. Ofloxacin has some activity against Gram positive and Gram negative anaerobic bacteria; however, high concentrations of the drug generally are required for in vitro inhibition and most of these organisms are considered resistant to the drug. This includes Clostridium and Bacteroides groups.

Pharmacokinetics

Ofloxacin is rapidly and almost completely absorbed from the GI tract following oral administration. The oral bioavailability is 85 to 100% in healthy, fasting adults, and peak serum concentrations are generally attained within 0.5 to 2 hours. Presence of food in the GI tract may decrease slightly its rate and/or extent of absorption, but this is not considered clinically significant. It is widely distributed into body tissues and fluids following oral administration and in healthy adults, the apparent Vd averages 1 to 2.5 L/kg. For most of the tissues and fluids into which ofloxacin distributes, its concentrations are about 0.5 to 1.7 times concurrent serum concentrations. Ofloxacin is concentrated within neutrophils, achieving concentrations in these cells that may be up to 8 times greater than extracellular concentrations. It is about 20 to 32% bound to serum proteins, mainly to albumins. Although less than 10% of a single dose of ofloxacin is metabolised, ofloxacin and its metabolites are excreted in both urine and faeces. Almost 80% of ofloxacin is excreted unchanged in urine and a small percentage of the dose is excreted in urine as metabolites and in faeces. In healthy adults with normal renal function, its elimination half-life in the terminal phase averages 4 to 8 hours. In healthy geriatric adults 64 to 86 years of age with normal renal function, half-life of ofloxacin averages 6 to 8.5 hours; although renal impairment will increase its serum concentrations and prolong its half-life.

(Reference: AHFS Drug Information 97)

Indication(s):

For the treatment of infections caused by susceptible strains of microorganism in infections of the urinary tract, lower respiratory tract, skin and soft tissue, gonorrhoea and non-gonococcal urethritis and cervicitis.

Consideration should be given to official guidance on the appropriate use of antibacterial agents.

Recommended Dosage:

1. Urinary tract infections: 200 to 400mg daily, preferably in the morning, increased if necessary in upper urinary tract infections to 400mg twice daily for 7 to 10 days.
2. Lower respiratory tract infections: 400mg daily, preferably in the morning, increased if necessary to 400mg twice daily for 7 to 14 days.
3. Skin and soft tissue infections: 200 to 400mg twice daily for 7 days.
4. Uncomplicated gonorrhoea: 400mg as a single dose.
5. Non-gonococcal urethritis and cervicitis: 400mg daily in single or divided

doses for 7 to 10 days.

6. Renal impairment: When creatinine clearance is 20 – 50mL/minute, the dosage should be reduced by half (100-200mg). If creatinine clearance is less than 20mL/minute, 100mg should be given every 24 hours. In patients undergoing haemodialysis or peritoneal dialysis, 100mg should be given every 24 hours.
7. Liver impairment: Dose of ofloxacin should be reduced accordingly in patients with impaired liver function.

Route of Administration: Oral

Contraindication(s):

1. Patient who has shown previous hypersensitivity to this drug or to other quinolone antibiotics.
2. Ofloxacin is contraindicated in patients with a history of epilepsy or with a lowered seizure threshold.
3. Ofloxacin is contraindicated in children or growing adolescents and in pregnant or breast-feeding women.

Side Effect(s) / Adverse Reaction(s):

Ofloxacin is generally well tolerated, and adverse effects of the drug are similar to those reported with other quinolone antibiotics. The most frequent adverse effects involve the GI tract or the CNS, including nausea, diarrhea, vomiting, abdominal pain/discomfort, abdominal cramps, flatulence, constipation, dyspepsia heartburn, decreased appetite, dry mouth, headache, insomnia, dizziness, nervousness and sleep disorders. Also reported rash and pruritus, tachycardia, transient hypotension, hepatitis, cholestatic jaundice, interstitial nephritis, eosinophilia, leukocytosis, anemia.;

Post Marketing Experience: Exacerbation of myasthenia gravis.

Musculoskeletal and connective tissue disorders*

Nervous system disorders*

General disorders and administrative site conditions*

Psychiatric disorders*

Eye disorders*

Ear and labyrinth disorders*

*Very rare cases of prolonged (up to months or years), disabling and potentially irreversible serious drug reactions affecting several, sometimes multiple, system organ classes and senses (including reactions such as tendinitis, tendon rupture, arthralgia, pain in extremities, gait disturbance, neuropathies associated with paraesthesia, depression, fatigue, memory impairment, sleep disorders, and impairment of hearing, vision, taste and smell) have been reported in association with the use of fluoroquinolones in some cases irrespective of pre-existing risk factors (see section Warnings and Precautions).

Psychiatric disorders

Rare: Depression (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)

Very Rare: Psychotic reactions (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)

Precaution(s) / Warning(s):

The use of ofloxacin should be avoided in patients who have experienced serious adverse reactions in the past when using fluoroquinolones containing products (see section Adverse Effects/Undesirable Effects). Treatment of these patients with ofloxacin should only be initiated in the absence of alternative treatment options and after careful benefit/risk assessment.

1. Patients on ofloxacin should be cautioned to avoid excessive exposure to direct sunlight while receiving the drug. If photosensitivity occurs (e.g. skin eruption), the drug should be discontinued.
2. As with other antibiotics, use of ofloxacin may result in overgrowth of nonsusceptible organisms, especially *enterococci* or *Candida*. Resistant strains of some organisms (e.g. *Pseudomonas aeruginosa*, *staphylococci*) have developed during ofloxacin therapy. Careful monitoring of the patient and periodic *in vitro* susceptibility tests are essential. If superinfection occurs, appropriate therapy should be instituted.
3. Ofloxacin should be used with caution in patient with renal or hepatic impairment because elimination of the drug is reduced in these patients.
4. Adverse effects may affect performance of skilled tasks (e.g. driving or handling machinery).

5. Dosage of ofloxacin does not need to be modified in geriatric patients with creatinine clearances greater than 50mL/minute.
6. The risk of developing fluoroquinolone-associated tendonitis and tendonrupture is further increased in people older than 60, in those taking corticosteroid drugs, and in kidney, heart and lung transplant recipients. Patients experiencing pain, swelling, inflammation of a tendon or tendon rupture should be advised to stop taking their ofloxacin medication and to contact their health care professional promptly about changing their antimicrobial therapy. Patients should also avoid exercise and using the affected area of the first sign of tendon pain, swelling, or inflammation.
7. Exacerbation of myasthenia gravis:
Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with fluoroquinolones use in persons with myasthenia gravis. Avoid fluoroquinolones in patients with known history of myasthenia gravis.
8. Aortic aneurysm and dissection:
Epidemiologic studies report an increased risk of aortic aneurysm and dissection after intake of fluoroquinolones, particularly in the older population. Therefore, fluoroquinolones should only be used after careful benefit-risk assessment and after consideration of other therapeutic options in patients with positive family history of aneurysm disease, or in patients diagnosed with pre-existing aortic aneurysm and/or aortic dissection, or in presence of other risk factors or conditions predisposing for aortic aneurysm and dissection (e.g. Marfan syndrome, vascular Ehlers-Danlos syndrome, Takayasu arteritis, giant cell arteritis, Behcet's disease, hypertension, known atherosclerosis).
In case of sudden abdominal, chest or back pain, patients should be advised to immediately consult a physician in an emergency department.
9. Prolonged, disabling and potentially irreversible serious adverse drug reactions:
Very rare cases of prolonged (continuing months or years), disabling and potentially irreversible serious adverse drug reactions affecting different, sometimes multiple body systems (musculoskeletal, nervous, psychiatric and senses) have been reported in patients receiving fluoroquinolones irrespective of their age and pre-existing risk factors. Ofloxacin should be discontinued immediately at the first signs or symptoms of any serious adverse reaction and patients should be advised to contact their prescriber for advice.
10. Tendinitis and tendon rupture:
Tendinitis and tendon rupture (especially but not limited to Achilles tendon), sometimes bilateral, may occur as early as within 48 hours of starting treatment with fluoroquinolones and have been reported to occur even up to several months after discontinuation of treatment. The risk of tendinitis and tendon rupture is increased in older patients (above 60 years of age), with renal impairment, patients with solid organ transplants, and those treated concurrently with corticosteroids in patients receiving daily doses of 1000 mg levofloxacin. Therefore, concomitant use of corticosteroids should be avoided.
At the first sign of tendinitis (e.g. painful swelling, inflammation) the treatment with ofloxacin should be discontinued and alternative treatment should be considered. The affected limb(s) should be appropriately treated (e.g. immobilisation). Corticosteroids should not be used if signs of tendinopathy occur.
11. Peripheral neuropathy:
Cases of sensory or sensorimotor polyneuropathy resulting in paraesthesia, hypaesthesia, dysesthesia, or weakness have been reported in patients receiving quinolones and fluoroquinolones. Patients under treatment with ofloxacin should be advised to inform their doctor and pharmacist prior to continuing treatment if symptoms of neuropathy such as pain, burning, tingling, numbness, or weakness develop in order to prevent the development of potentially irreversible condition (see section Adverse Effects/Undesirable Effects).
12. Psychiatric reactions:
Psychiatric reactions may occur even after the first administration of fluoroquinolones, including Ofcin F.C. Tablet. In rare cases, depression or psychotic reactions can progress to suicidal ideations/thoughts and self-injurious behaviour, such as attempted or completed suicide (see section 'Side Effect(s) / Adverse Reaction(s)'). In the event that the patient develops these reactions, Ofcin F.C. Tablet should be discontinued and appropriate measures instituted. Caution is recommended if Ofcin F.C. Tablet is to be used in psychotic patients or in patients with a history of psychiatric disease.

Pregnancy and Lactation:

Ofloxacin is contraindicated in pregnant or breast-feeding women.

Drug Interactions:

Clinically important drug interactions of ofloxacin include those with antacids, sucralfate, oral antidiabetics, warfarin and theophylline. Both antacids and sucralfate decrease absorption of oral ofloxacin; effects of oral antidiabetics may be enhanced by ofloxacin; effects of warfarin on prothrombin time and theophylline level may be increased by ofloxacin. With high doses of quinolones, impairment of excretion and an increases in serum levels may occur when co-administered with other drugs that undergo renal tubular secretion e.g. Probenecid. Other drug interactions include those with aminoglycosides, beta-lactam antibiotics and antimycobacterial agents.

Symptoms and Treatment for Overdosage, and Antidote(s):

Overdosage of ofloxacin would be expected to produce manifestations that are extensions of the adverse reactions reported with the drug, and may include nausea, vomiting, seizures, vertigo, dysgeusia and psychosis.

If acute overdosage of ofloxacin occurs, the stomach should be emptied by inducing emesis or by gastric lavage. Supportive and symptomatic treatment should be initiated, and patient should be observed carefully; while adequate hydration should be maintained. Because ofloxacin is not efficiently removed by haemodialysis or peritoneal dialysis, these procedures should not be relied on to enhance elimination of the drug from the body.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):

Ofcin F.C. Tablet 200mg

A slight-orange color film coated round tablet

Blister packing of 10's x 10 and 10's x 50

Ofcin F.C. Tablet 400mg

A slight-orange color film coated elliptical tablet, one side impressed with a score “   ”

Blister packing of 10's x 10 and 10's x 50



Manufacturer and Product Registration Holder:
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